



Mental Health Patient, Carer and Visitor Support Post-Critical Incident SOP

Scope

Site	Department, division, or operational area	Applicable to
Royal Perth Bentley Group (RPBG)	Mental Health Division (MHD), Inpatient wards and services, Community Mental Health Services (CMHS) and Safe Haven	All staff

Purpose

This standard operational guideline (SOP) outlines requirements to ensure mental health patients and, if appropriate, their carers, personal support persons (PSPs), and visitors receive the best support, information, and care, following a critical incident.

Patients and consumers are terms used interchangeably; this SOP will refer to “patients”.

Definitions

After-hours cover	Consists of an after-hours mental health (MH) Clinical Nurse Specialist (CNS) and Duty Medical Officer (DMO) who report to the Duty Psychiatrist and CMHS Assessment and Treatment Team (ATT).
Care coordinator	The nominated member of the community multidisciplinary team (MDT) who has responsibility for coordinating, facilitating, and integrating MH treatment, care, and support for a MH consumer
Community treating team	An MDT which may comprise staff from psychiatry, psychology, nursing, occupational therapy, social work, pharmacy, and other relevant groups who collaborate in the delivery of care for MH consumers.
Critical incident	An unexpected, sudden, and traumatic event, outside of the usual range of daily experiences involving a personal threat, which evokes extreme stress, fear or injury and affects one's sense of security, e.g., an assault, threat of harm, witness to an event that causes someone harm, severe injury, death (actual or threatened), serious accident or natural disaster.
Critical incident stress debriefing (CISD)	A brief crisis intervention, session, group debriefing, usually administered within days of a critical incident or traumatic event.
Defusing	Another component of CISD, allows for the ventilation of emotions and thoughts associated with the critical incident/crisis. Ventilation and validation are necessary to give the individual an opportunity to emote ^{1,2} .

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First responder	The first person to arrive at the scene who responds to the critical incident or emergency situation and stops the incident from progressing to more dire consequences and/or provides initial clinical management in the environment.
Inpatient treating team	Consists of ward or service coordinator, Nurse Unit Manager (NUM), CNS, Consultant Psychiatrist and Registrar.
Near miss	An event or situation that could have resulted in an accident, injury, harm, or illness, but did not, either by chance or through timely intervention.
Post-acute period	Begins once the critical incident calms or normalises and a person may be at risk of continued distress, because of lower resilience and/or other significant life stress and can lead to diagnoses of PTSD, major depression, alcohol/other substance use disorders and grief reactions ³ .
Post-traumatic stress disorder (PTSD)	Involves the emotional and physiological distress (e.g., intense fear or helplessness) experienced by people during or after an event that threatened the life, safety or physical integrity of oneself or others.

Background

Impacts of critical incidents

Critical incidents have the capacity to adversely affect individuals and persons close to them. This could be by directly experiencing a critical incident or near miss; witnessing an incident; or learning that the event occurred to a close family member or friend¹.

Common reactions to critical incidents can include physical, psychological, and emotional symptoms¹. Many individuals experience symptoms of distress because of a critical incident and recover from these symptoms. However, some may develop psychiatric sequelae, for example, an acute stress disorder or PTSD³.

Patients with an increased risk of developing complications after a critical incident include those with:

- prior trauma histories, experiences, or intergenerational trauma
- prior MH issues/other disabilities
- family psychiatric history
- perceived threat to life
- poor social support
- culturally diverse backgrounds (e.g., Aboriginal people, LGBTIQ+ persons)
- past history of PTSD^{3, 1}

Background cont'd

Support for post-critical incidents

1. Support should be provided to individuals as voluntary, one to one support sessions, in the immediate period following a critical incident and ongoing, as required.

Psychosocial supports that are recommended include:

- reducing heightened emotional, psychological, and physical arousal where possible, as appropriate
- support to safely express emotions
- support to mobilise coping skills
- identifying suitable supports either existing or via referrals for professional support
- attending to practical concerns
- the continuation of emotional and practical support, as required, for individuals whose natural recovery is historically slow ^{3, 4, 5}

All assessment and treatment is to align with trauma informed care (as per [Chief Psychiatrist's Standards for Clinical Care](#) and [Charter of Mental Health Care Principles \(Mental Health Act \[MHA\] 2014\)](#)).

2. Refer patients to Service Allied Health staff or Clinical Psychology Services to determine if additional support is required. Clinical Psychology and Allied Health staff will outsource if necessary. For information about the referral process, refer to [Referral to counselling and support services](#) section.

For patients with a past or current history of PTSD, psychological interventions may be considered after a period of stabilisation. Only evidence-based interventions should be used as a part of a comprehensive treatment plan and conducted by a person with specialised training.

Note: The literature supports one-off or a series of individual support sessions. Group debriefing such as critical incident stress debriefing (CISD) (a crisis, group intervention) is no longer recommended for either patient, carers, staff, etc. ^{1,3,5}.

Procedure details

Staff response: dealing with the immediate situation post-critical incident

The relevant managers (e.g., nursing, medical lead, or Allied Health) are to:

- provide staff with as much preliminary or preparatory information about the critical incident, as possible
- ensure staff are supported in accordance with the requirements of the **EMHS Critical Incident Stress Debriefing Policy** including the Employee Assistance Program (EAP)
- consider follow up appointments to gauge further support requirements
- see [Reporting post-critical incidents process](#) section.

Staff (i.e., Shift Coordinator, NUMs, senior nursing, treating team and/or team leaders) are to determine allocated lead clinicians to provide the following responses across the different timeframes.

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Procedure details cont'd

Immediate response (0 – 8 hours)

Relevant to all individuals involved initially, but patients only longer term.

For patients, PSPs and visitors affected by a critical incident, first responders and/or allocated lead clinicians are to immediately:

- alert all relevant staff to provide assistance with the affected patients, PSPs and/or visitors, as appropriate
- assess immediate reactions to the event on the patient, PSPs and/or visitors and

acknowledge any distress experienced

- check the immediate welfare and risks, and ability of the patients, PSPs, and/or visitors affected to keep safe in the immediate aftermath. In the event of an alert to evacuate (both in the inpatient and community setting), refer to the **EMHS Emergency and Disaster Management Policy**
- ensure strategies are implemented for the care of the patients, PSPs and/or visitors as dictated by the nature of the incident, e.g., first aid for physical injury
- provide information about patient rights and/or responsibilities and on process being followed, including time frames, as appropriate (refer to the **R PBG Consumer, Family and Carer Information SOP**)

Escalation

- Notify the patient's relevant treating team (community, inpatient or after-hours)
- Inform and escalate to relevant senior line managers (NUMs, Team Leaders, Program Managers, Medical Clinical Lead, Senior Nurse on Call (SNoC) and Duty Psychiatrist, where relevant) that a critical incident has occurred as soon as practicable, however, no later than by the end of the notifying staff's working day and/or shift
- For serious staff assault report the incident to the Executive On Call or Co-Directors, as relevant
- Liaise with external authorities, as needed, e.g., WA Police.

Short-term response (24 - 48 hours)

Debrief

For patients, PSPs, and visitors, where appropriate, affected by a critical incident, the allocated lead clinicians are to:

- provide brief, individual debrief sessions, offered on a voluntary basis within the first 24 hours after the incident, including provision of (as appropriate):
 - a short factual statement about what is known about the incident (causes and consequences)
 - assistance to the patients, PSPs, and visitors where appropriate to understand and manage their preliminary reactions to the event, including any memories triggered by the incident. Refer to principles of trauma informed care
 - information to patients, PSPs, and visitors where appropriate about how they can care for themselves and reinforce coping strategies^{2,6}

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Procedure details – Short term response (24 - 48 hours) cont'd

- use open disclosure principles to guide defusing and discussion with patients, PSPs, and visitors as per the **WA Department of Health Open Disclosure Policy**.

Risk Assessment

The allocated lead clinicians are to:

- Review and update the patient's *Risk Assessment Management Plan (RAMP)* and, with consent, involve the patient's PSPs, as appropriate
- The assessment is to include present or history of exposure to abuse or critical incidents as part of a developmental history, along with other vulnerability
- Update the patient's Psychiatric Services Online Information System (WebPSOLIS or PSOLIS) *Treatment and Support Discharge Plan (TDSP)*, *Outpatient Management Plan*, *Stay Strong Plan* - including the *Crisis Awareness Care Plan* and/or *Delivers What Matters Most Record (DWMMR) (Inpatients)* (Refer to the **RFBG Comprehensive Care SOP** and **Community Mental Health Services Triage to Discharge SOP**)
- Activate a PSOLIS alert, as appropriate
- Complete iSoBAR handover information and include an alert on the patient's journey board, if appropriate, and discuss risks at inpatient or CMHS review meetings and relevant meetings.

Medium-term response (days after the incident)

Support for post-critical incidents

Staff (i.e., Shift Coordinator, NUMs, Team Leaders, senior nursing, treating team) are to consider the following support strategies to assist patients, PSPs, and visitors where appropriate, post-critical incidents, especially if the patient is initially deemed as at risk or is highly symptomatic, including:

Support sessions

- Facilitate support sessions after the incident, as appropriate, and this may include individual or more brief, informal sessions, offered on a voluntary basis, and progressed as below.

Procedure details – Medium-term response (days after the incident) cont'd

Referral to counselling and/or support services

- Offered on a voluntary basis to patients, PSPs, and visitors where appropriate who have identified vulnerability. For Safe Haven, support sessions are not appropriate and will require referral to counselling and/or support services
- For patients, PSPs, and visitors where appropriate, (and as requested) are offered brief, supportive contact with the MHD's Service Allied Health staff – e.g., social workers or clinical psychologists. Clinicians are to:
 - email a referral request to the service inpatient clinical psychologist and Head of Department (HoD) Clinical Psychology and Clinical Neuropsychology, or Allied Health team for patients, PSPs, and visitors where appropriate. Face to face referral request may also be made at team reviews (with this followed up in an email). An appointment is to be offered by the above, or as delegated, within 2 working days of the email or face to face referral request as appropriate (if this timeline cannot be met, it should be escalated through the appropriate line management). Refer to **eReferral Process: Inpatient and Outpatient SOP**
 - Refer to RPBG community Clinical Psychology services through the existing process or to other, psychological counselling supports and emergency helplines for patients and PSPs of CMHS, as required. Referrals are made face to face in team reviews and followed up by email
 - Make referrals to carer support services and/or other supports for patients' PSPs. Refer to **Visitor Management Policy**.

Patients at risk of post-acute symptoms

- For those patients where appropriate with identified post-acute signs and symptoms of more complex needs during the days following the incident, the clinician and treating team are to:
 - continue support sessions, as appropriate
 - review and update the patient's *RAMP* at approximately 4 weeks or as indicated
 - review and update PSOLIS Alerts and the patient's relevant PSOLIS *TSDP*, *DWMMR*, *Crisis Awareness Care Plans*, as appropriate
 - Discuss risks at inpatient or multidisciplinary review meetings and at clinical handover
- Consider the use of the *Progressive Risk Assessment (PRA)*, if appropriate (inpatient areas)
- Consider a need for long-term psychological treatment or interventions for the patient's care and provide direction and information/referral. Refer to **Recognising and Responding to Acute Deterioration in Mental State, Cognition and Behaviour SOP**
- If discharged or the patient no longer engages with a service, establish a follow-up and referral mechanism for the patient, PSPs, and visitors where appropriate – refer to general practitioners (GPs) and/or notify GPs of the clinical incident via *Notifications and Clinical Summaries (NaCS)* and *Care Transfer Summary* or letter, regardless of other follow up in place

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Procedure details – Medium-term response (days after the incident) cont'd

- Closely monitor and regularly assess throughout the patient's care and at times of transition, such as movement to high or lower dependency areas, imminent discharge or within the CMHS.

Reporting post-critical incidents

As soon as practicable, lead clinicians and treating team are to:

- Document all critical incidents in the patient's healthcare record with first observation or knowledge of the incident and the clinical care, including:
 - defusing and/or support session(s)
 - participation of the patient and/or PSP in their care plan and responses
 - completing a Datix Clinical Incident Management System (CIMS) and/or a Combined Hazard or Incident Reporting (CHOIR) Online Form **within 48 hours** of the incident. Refer to the **WA Department of Health CIM Policy**
 - notifying the Chief Psychiatrist of any incidents as per the **Policy for Mandatory Reporting of Notifiable Incidents to the Chief Psychiatrist**
 - entering service event into PSOLIS
 - reviewing and updating all statewide standardised clinical documentation (SSCD) and National Outcomes and Casemix Collection (NOCC) outcome measures and document in the patient's healthcare record. Refer to the **WA Department of Health SSCD Guideline**
 - notifying senior staff of the event.

The manager's role after a critical incident

Immediate actions following an incident

- The relevant manager (e.g., nursing or medical lead, Team Leader or Allied Health):
 - ensures that any patients, PSPs and/or visitors affected by an incident are safe and assists with all necessary steps to support the individuals involved to prevent further injury from occurring
 - consults with patients, PSPs, and witnesses to capture sufficient information and evidence (if applicable) to assist with later review as per the **WA Department of Health CIM Policy**
 - ensures staff have completed a debriefing with the patient and PSP involved and where appropriate
 - ensures staff receive debriefing and support as per **EMHS Critical Incident Stress Debriefing Policy** and/or via third party contractual arrangements, where relevant.

Compliance monitoring

Compliance will be monitored during the course of clinical incident management and CHOIR investigations.

Facilitator

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Aboriginal Health Impact Statement and Declaration

The voice of the Aboriginal community is reflected in the Mental Health Patient Consumer, Carer and Visitor Support Post-Critical Incident SOP through consultation with Aboriginal Health Strategy Team to ensure that cultural appropriateness and the health impacts on Aboriginal people have been considered and incorporated (ISD Record 2482).

Note: Within Western Australia, the term Aboriginal is used in preference to Aboriginal and Torres Strait Islander, in recognition that Aboriginal people are the original inhabitants of Western Australia. No disrespect is intended to our Torres Strait Islander colleagues and community.

Related policies, practice standards, clinical guidelines

R PBG Policies

- [Consumer, Family and Carer Information SOP](#)
- [Mental Health Inpatient Triage to Discharge SOP](#)
- [Comprehensive Care SOP](#)
- [eReferral Process: Inpatient and Outpatient SOP](#)
- [Adult and Older Adult Community Mental Health Services Triage to Discharge SOP](#)
- [On Duty Clinician Community Mental Health Services SOP](#)
- [Clinical Handover Policy](#)
- [Clinical Care of People Who May Be Suicidal Policy](#)
- [Sexual Safety Policy](#)
- [Save Haven Access and Provision of Care SOP](#)
- [Recognising and Responding to Acute Deterioration in Mental State, Cognition and Behaviour SOP](#)
- [Recognising and Responding to Acute Deterioration Policy](#)
- [End of Life CPS](#)
- [Visitor Management Policy](#)

EMHS Policies

- [Post Incident Management / Critical Incident Stress Debriefing Policy](#)
- [Care Coordination in Mental Health Services Policy](#)
- [Acute Response Mental Health Emergencies / Crisis Policy](#)
- [Emergency and Disaster Management Policy](#)
- [Prevention and Management of Workplace Aggression and Violence Policy](#)
- [Rights of Patients in Mental Health Services Policy](#)
- [Rights of Carers and Other Personal Support Persons Policy](#)
- [Open Disclosure Policy](#)

WA Department of Health Policies

- [Recognising and Responding to Acute Deterioration Policy](#)
- [Safety Planning for Mental Health Consumers Policy](#)
- [Principles and Best Practice for the Care of People Who May Be Suicidal](#)
- [Safety Planning Procedures for Mental Health Consumers](#)
- [Principles and Best Practice for the Care of People Who May Be at Risk of Exhibiting Violent or Aggressive Behaviour](#)
- [Workplace Aggression and Violence Policy](#)
- [Clinical Incident Management \(CIM\) Policy](#)
- [Statewide Standardised Clinical Documentation \(SSCD\) Guideline](#)

Other

- [Chief Psychiatrist's Standards for Clinical Care 2015](#)
- [Charter of Mental Health Care Principles \(MHA 2014\)](#)
- [Policy for Mandatory Reporting of Notifiable Incidents to the Chief Psychiatrist \(Public Mental Health Services\)](#)

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Related forms

- *Care Management Plan Outpatients*
- *Delivers What Matters Most Record (DWMMR) (Inpatients)*
- *Crisis Awareness Care Plan*
- *Risk Assessment Management Plan (RAMP)*
- *Mental Health Treatment and Support Discharge Plan (TSDP)*
- *Care Transfer Summary*
- *Notifications and Clinical Summaries (NaCS)*

Related national standards

ACSQHC NSQHS Standards 2nd Edition (2021); Standard 1: Clinical Governance; Standard 2: Partnering with Consumers; Standard 6: Communicating for Safety
NMHS Standards (2010); Standard 1: Rights and responsibilities; Standard 2: Safety; Standard 6: Consumers; Standard 7: Carers.
Australian Charter of Healthcare Rights Guiding Principles: Safety and Respect.

References

1. Cristian Vasile. (2014). *An analysis of psychological trauma interventions*. Procedia - Social and Behavioral Sciences 127, 781 – 785.
2. Department of Health & Ageing. Government of South Australia. (Oct 2017). *Challenging Behaviour Toolkit; Tool 6: Guide to reporting and review of behaviour incidents*. South Australia.
3. Craig L. Katz. (2017). *Best Practice: Mental Health response to disasters and other critical incidents*. BMJ Publishing Group Ltd, New York.
4. Canadian Agency for Drugs and Technologies in Health. (2015). *Treatment for Post-Traumatic Stress Disorder, Operational Stress Injury, or Critical Incident Stress: A Review of Guidelines*. Ottawa. Canada.
5. Thomas, J. (2018). *The 7 Steps Of Critical Incident Stress Debriefing And How They Support Trauma Recovery*. Better Help, California.
6. Mark Techmanski. (2013). *Critical Incident Stress Management and Peer-Support For Law Enforcement Personnel: An Objective Review of Data and Practice for Administrators*. The Journal of Law Enforcement, Volume 3, Number 2. ISSN: 2161-0231 (Online).